It is a traumatic time when you are first told your child needs a stoma. Surgery may need to be carried out urgently, often within hours of your baby's birth, and there may be little time for consultation and discussion. You will find that hospital staff is caring and supportive, but you may not have been able to take in all the information you need at once.
Never be afraid to ask for help, advice and explanations. This booklet aims to give your additional practical advice, reassurance and information about the day-to-day care of a baby or child with a stoma - whether it is created as a temporary or permanent measure – and to answer the many queries that are bound to arise.

What is a stoma?
The words stoma and Ostomy come from the Greek word meaning 'mouth' or 'opening'. Approximately half inch of the bowel is brought to the surface of the abdominal wall and turned inside out before being attached with small stitches which usually dissolve on their own. Stool passes out of the stoma into a special stoma bag/pouch in a controlled manner.

There are three types of stoma

Colostomy
An opening made into the large intestine (colon)

Ileostomy
An opening made into the small intestine (ileum)

Urostomy
An opening made to divert urine from the bladder

Stomas look like a small bud or raspberry and most resemble the inside of the mouth in colouring. The colour can fluctuate between pale and dark red as the muscles of the bowel contract and relax. This change in colour is normal and should not worry you unless the colour goes very dark and does not fluctuate. Stomas can bleed quite easily but this does not cause pain as there is no feeling in the stoma. The bleeding usually stops rapidly as it starts. If you are at all worried about the look of your child's stoma do not hesitate to discuss it with your doctor or stoma care nurse.

Changing Pouches
Before changing a pouch, make sure that you have the new one ready and everything you need at hand. With a baby, this is just like changing a nappy, only you seem to need more hands to hold the baby still, stop tiny hands from getting in the way, and to position the pouch making sure it is on properly with no leaks.

Removing a used pouch
Empty the pouch first, and then carefully peel it off with one hand while gently pressing down on the skin with the other. Seal the used pouch in a plastic bag or nappy sack for disposal in your bin.

Never flush used Pouches down the toilet

Cleaning the stoma area
Clean the stoma and surrounding area using warm water and gauze wipes. Cotton wool balls are not recommended because they can leave strands on the skin which can cause leakages. The skin should be

washed gently, not rubbed, and if soap is used it must be rinsed off well. Dry the skin thoroughly, patting gently – do not rub.

Fitting the new pouch
When applying the pouch do not forget to check it fits snugly around the stoma and that the stoma size has not altered.

Getting Your Pouches
You can obtain your supplies from the hospital pharmacy. Always take the special card and prescriptions.
Pouches should not be exposed to strong sunlight, airing cupboards or other hot places as they tend to deteriorate.

Baths and Showers
Stoma pouches can be left in place or removed whilst bathing depending on personal choice. Many parents decide to bath their child when the pouch needs changing.

Diet
A normal balanced diet is recommended. Some foods produce more wind than others, so perhaps it is best to avoid these where possible. Wind and odour are the main problems and trial and error will soon enable you to discover which foods are best avoided.

<u>Ileostomy</u>
Some foods which are hard to digest such as tomato skins and celery may be best avoided. Children with ileostomies will demand more to drink than normal. This is only their way of replacing lost fluids. Drinking more will not make the waste any more fluid, but helps the kidneys to work properly.

<u>Urostomy</u>
Children with urostomies should be given plenty of clear liquids to keep the urine dilute. This helps to protect the kidneys from infection.

Clothing
Young babies are easy to dress with stretch one piece suits, dungarees and little dresses. As your child gets older, try to avoid stiff waistbands, belts or tight and clinging fabrics. Jogging suits are popular and practical, providing the elastic waist is not too tight. Bold patterns and baggy T-shirts are recommended. However modern pouches are carefully designed to be flat, odour free and rustle free and are quite unnoticeable under ordinary clothes.

Swimming and Sport
Unless your doctor has told you otherwise there is no reason why your child cannot swim or take part in most sports, although, as your child grows up, contact sports such as rugby are best avoided.
If your child swims a lot he or she may require more frequent pouch changes, but this should not prove a deterrent to fun and good exercise.
Shorts and matching T-shirts can often be worn on the beach. Girls who like swimming can wear patterned swimsuits and boys can wear swimming shorts that go up to their waist.

Travelling
Travelling can be managed quite easily. It is advisable to pack double the amount of pouches than you would normally use. A large waterproof toiletry bag is very useful for this and should contain:

- The pouch plus a spare
- A small amount of water for cleaning
- Gauze wipes

- A small plastic bowl
- Any skin preparation, barrier wipes etc, that you use
- A pair of scissors
- Plastic disposable bags
- Hand wet wipes

After a time you will get to know how often the pouch will need changing, and you can usually avoid having to do this outside the home. Never rely on this though. A travelling kit can often save the day, and it takes so little space that it is worth carrying for the peace of mind that it gives.
Never delay changing a leaking pouch for any reason. It only takes a short while for the skin to become sore. The equivalent of a bad case of nappy rash on the tender skin of a baby's stomach is to be avoided at all costs.

Informing others
You will decide as to who you want to inform about your child's stoma and what you wish to tell them. Obviously anyone who will be looking after your child will need to be given practical information about how to look after your child's stoma.
It can be a good idea to discuss your plans with your stoma care nurse or a trusted friend.
As your child gets older you can discuss giving explanations with them. It is usually a good idea to have a simple explanation ready for answering other children's questions, as once children's curiosity is satisfied they lose interest more quickly than if an atmosphere of secrecy prevails.

Problems which may arise

Skin soreness
Leakage is the most common cause of skin soreness. Try to avoid this by making sure the pouch fits well around the stoma. Always ensure waste matter does not come into contact with the surrounding skin and thoroughly clean the skin with every pouch change.
Removing the pouch too frequently (or too harshly) can cause problems. Try not to change a pouch unnecessarily – babies and children can be bathed quite successfully without removing the pouch.
Another cause of skin soreness can be allergies to adhesives, soaps or creams etc. Use warm water for cleaning and avoid using harsh soaps.
Skin soreness is much easier to prevent than cure. Once a child's skin becomes sore, it may become difficult to get the pouch to adhere to the skin – thus increasing the likelihood of leaks and making the skin even worse. Try to clear up skin problems as soon as possible – the longer they last, the harder they can be to cure. Always seek help from your doctor or/and stoma care nurse.

Bleeding or change in stoma size
Surface bleeding of the stoma is quite normal and nothing to worry about. Bleeding can happen when washing or when a pouch is fitted too tightly and irritates the stoma. Cleaning more gently or adjusting the fitting of the pouch will help to stop bleeding.
Bleeding from inside the stoma is not normal and this, or any sudden change in stoma size, should be reported to your doctor.

Diarrhoea or constipation
There is no reason why a child with a stoma should experience diarrhoea or constipation anymore than anyone else.
If your child is constipated make sure they drink plenty of fluids and, if weaned, give them fruit juice and fruit purees. (In a child with ileostomy constipation presents as waste matter being too thick)
Some foods do have a tendency to cause diarrhoea. With a little detective work you can soon discover the culprit and adjust your child's diet accordingly. It is also important for a child with diarrhoea to drink plenty of fluids as diarrhoea can cause considerable loss of salt and water leading to dehydration. A baby with diarrhoea can become very ill, very quickly. If in doubt consult your doctor.

Certain medicines e.g. antibiotics can sometimes give rise to constipation or diarrhoea. Keep a close eye on your child if they are on a new medication. If you notice any changes contact your doctor.

Odour and wind
Modern pouches are designed to be odour proof and odour should only occur when changing. There is a range of deodorants on the market now, some to be used inside the pouch or some as sprays.
Wind can be caused by certain foods but this will be individual to each child. If wind is a problem you should find which foods to avoid by trial and error. For the older child yoghurt may help.

Prolapse
Sometimes the muscles holding the stoma weaken and allow the bowel to slide out, increasing the length of the stoma. Although it can look quite frightening prolapse does not usually cause any pain or affect the way the stoma works. This may happen after a bout of coughing or crying, and will sometimes remedy itself after a period of rest, but the doctor should be informed.

Retraction
This is the opposite of prolapse. The stoma can sink below the level of the abdominal skin. This makes it difficult for the pouches to stay on as the waste can leak more readily under the flange. If the stoma is functioning well do not worry but inform the doctor.

Rectal discharge
This happens when some of the waste or yellowish mucous which lubricates the bowel passes through the rectum. Unless the discharge is offensive there is nothing to worry about. On these occasions children who are not wearing nappies can use incontinence pads.

Just as a reminder

- Never rush. Take time to cleanse properly as good skin care is the key to success.
- Prepare everything in advance.
- Watch your stock levels. Never let yourself have less than a week's supply.
- Once a pouch leaks change it as soon as possible.
- Have a list of all the people and their phone numbers who are involved in your child's care

Reference: CliniMed Limited

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